

العمر (تاريخ الميلاد) : 12-12-1977 المهنة : لا تعمل

رئيس
المجلس الطبي المتخصص

Name: هناء حبيب عيسى عطيه
National ID: 27712122600841



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من بشأن تقرير طبي للمريض:
بقسم رقم طبي

تتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ رساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد انه:

وتم استرجاعه الى مسكنه
على ما هو عليه

تحريراً في

التاريخ : ١٦/٢٩

الوقت:

الطبيب المعالج
د. محمد

يعتمد،،

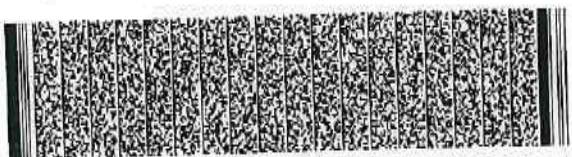

جمهورية مصر العربية
 بطاقة تحقيق الشخصية

هنا
حبيب عيسى عطيه
 ٢ ش ثابت ميخائيل - كمال حمزة - العنيزة
 امبابه - الجيزه

١٩٧٧/١٢/٢٦ ٢٧٧ ١٢ ١٢ ٢٦ ٠٠٨ ٤١
 LJ9358382


 ٢٠٢٥/٠٩/٢٨ ٢٧٧ ١٢ ١٢ ٢٦ ٠٠٨ ٤١
 حاصله على دبلوم تجاره
 انشى مسيحية متروحة
 ميلاد ثابت ميخائيل خليل
 البطاقة سارية حتى ٢٠٣٢/٠٩/٢٨





04/20/2026

MDT FORM

Name:	Hana Habieb Essa .	MRN:	462
Age:	48 Y	ID:	27712122600841
Menopausal Sta	premenop	Phone:	01225947395
NRS SCORE:	ps 0	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: Idc
ER: 8/8 PR: 8/8 HER2: 3+ KI67:

R

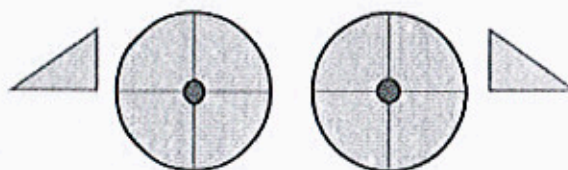
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Sonomammography:Mets Workup:History:

premenop recurrent left breast cancer idc er8/8 pr8/8 her23+ .history 2019 left bcs did not receive tt 2025 recurrence left mastectomy 9/2025 pt2 nx mo by pet ct rec 6 taxotere carboplatin for mdt for ht ef 66 % 15/4/2026 for us breast.for c5 herceptin adj 600 sc for mdt for adjuvant hormonal tt

Recommendations:MDT Outcome & Plan

continue ADJUVANT HERCEPTAN till one year +AI +Zoladex
PORT if not previously received



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نرمن مصطفى		د/ ماهر مختار			

Patient Name : الأستاذة / هناء حبيب عيسى	Request No. : 13006793
Patient No. : 27712122600841	Request Date : 16/10/2025
Path No. : SR-1440/25	Verification Date : 21/10/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 47 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).
- Ki67%: 60%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 01/11/2025 10:26:46

Page 2 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / هناء حبيب عيسى	Request No.	: 13006793
Patient No.	: 27712122600841	Request Date	: 16/10/2025
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Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Referred blocks & slides (Left mastectomy).

Gross Examination:

- Six referred blocks labelled by (3719-25). Together with, five immunostained slides for (CK, ER, PR, HER2 & Ki67).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * DCIS: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Skin: Free.
- * Surgical margin: Free.

* IPT:

- CK: Diffusely strong positive staining in tumor cells.
- ER: Strong nuclear staining in 80% of tumor cells (score 8/8).
- PR: Strong nuclear staining in 95% of tumor cells (score 8/8).
- Her2: Strong complete membrane staining in more than 10% of tumor cells (score 3+).
- Ki67%: 60%.

Diagnosis:

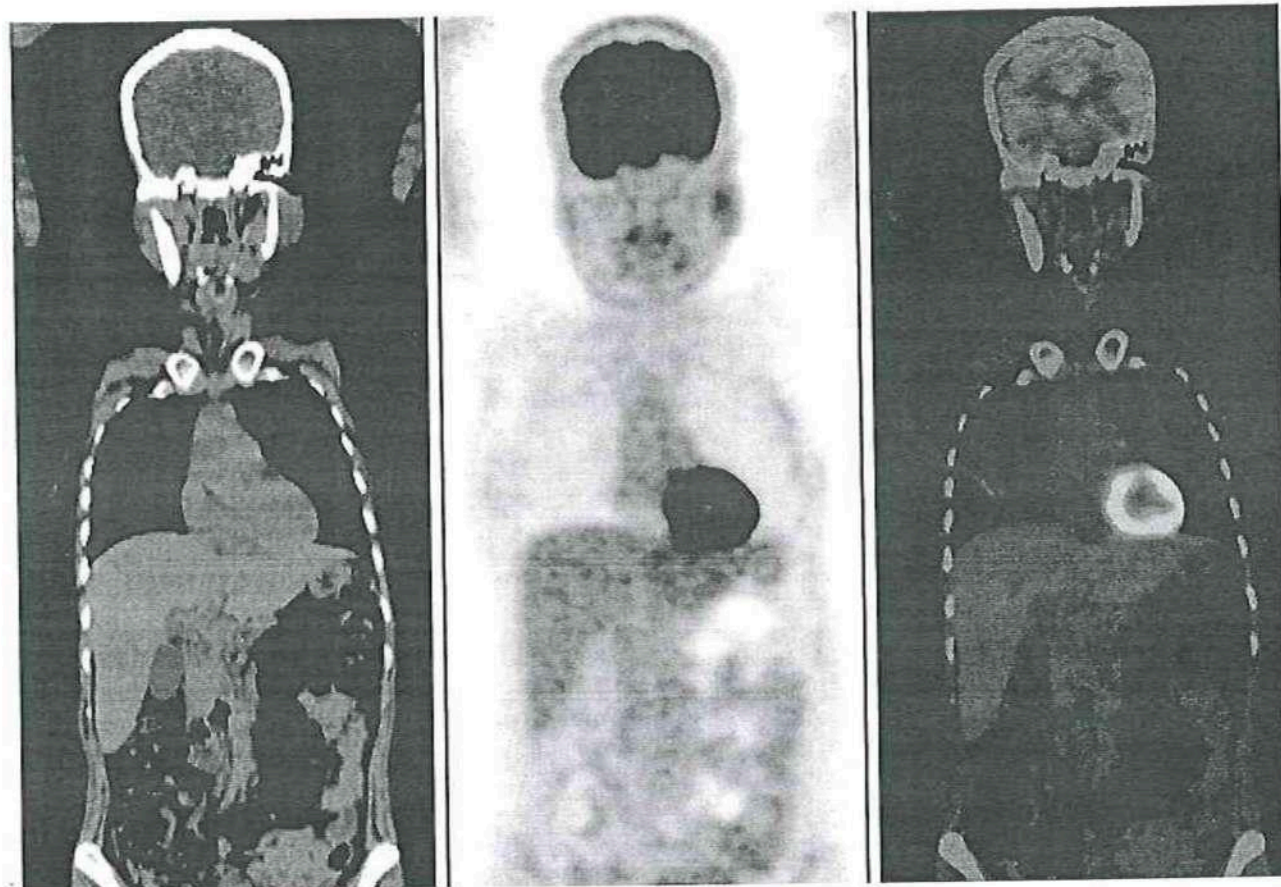
Left breast cancer, Referred blocks & slides (Left mastectomy):

- Invasive duct carcinoma, Grade 3.
- Skin: Free.
- Surgical margin: Free.

Verified by :

Dr.Duaa Mohamed





IV. Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses.
- No metabolically active (FDG avid) hilar or mediastinal lymph-nodes.
- No metabolically active (FDG avid) pleural or pericardial effusion.
- Normal appearance of chest wall with no related FDG avid lesions

V. Abdomen and pelvis:

- Enlarged liver showing no metabolically active (FDG avid) focal hepatic lesions.

Echocardiography report

Patient name	هناء حبيب عيسى	Date	25/9/2025
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M- mode:

IVSD	1.2	AO	2
PWD	1.2	LA	3
LVIDD	4.3	FS	30%
LVIDS	2.8	EF	65%

Comments:

- Left ventricle:
 - ✓ Normal systolic function and normal internal dimension
 - ✓ diastolic dysfunction grade 1
 - ✓ No RMWAs :
- Right side: normal
- Left atrium: normal
- Mitral valve: Normal
- Aortic valve: Normal
- Tricuspid valve: normal
- Pulmonary valve: Normal
- No intracardiac masses nor shunts.
- No pericardium effusion

Conclusion:

DD1

Dr. Faris Elwakeel

Cardiologist, M.Sc

NHI



NAME: Mrs. Hanae Habib

Date : 25/8/2025

Ref prof Dr Sami Muffied

Bilateral Ultrasound for the Breast Revealed:**I. Clinical status :**

Left conservative mastectomy .

Left breast lump .

II. Findings:

- Dense fibroglandular architecture of the breasts. (ACR 3)
- An irregular speculated soft tissue mass lesion is seen in the retroareolar region of the left breast at 12 o'clock with thin long tentacles radiating from its outer margins causing focal parenchymal distortion of hypoechoic solid texture of the mass lesion measuring 18.4 x 12 mm in diameter .
- Color Doppler imaging revealed increased peripheral vascularity of the mass lesion with high RI = 0.73 .
- Mildly enlarged multiple left axillary lymph nodes measuring 19 x 7.8 mm in diameters with hypertrophied cortex .
- Bilateral scattered breast cysts measuring 5-7 mm in diameters .

CONCLUSION:-

- Findings are those of neoplastic mass lesion of the retroareolar region of the left breast . (BIRADS 4c) .
For US guided core needle biopsy .
- Fibrocystic disease of the breasts .

Definition of Breast Imaging Reporting and Data System (BIRADS)

- 1) Normal
- 2) Benign
- 3) Probably benign
- 4) Probably malignant
- 5) Malignant

Dr. Manal Halim, MD

Prof of radiodiagnosis . Cairo University

المركز الرئيسي / ٢٨ شارع روض الفرج - شبرا - القاهرة

تليفون: ٢٤٣٢٠٨٥٠ - ٢٤٣٠١٢٩٧ - ٢٤٣٠١٢٩٠ فاكس: ٢٤٣٢٣٨٥٥ موبايل: ٠١٢٨٩٣٠٢٢٦٦

فرع المهندسين / بالمستشفى الدولي للكلية والمسالك البولية

٤ شارع عدنان المدني من شارع السودان - المهندسين / تليفون: ٣٣٠٥٢٠٠٠ موبايل: ٠١٠٠٨٤٤٣٣٦٩

Omegascan.sh@gmail.com

Patient's Name: Mrs/ *Hanaa Habib Eessa*
Date; 30/9/2025

Whole Body PET/CT Examination

Clinical history:-

- A 48 years old female, gave history of left breast carcinoma underwent left mastectomy at 2025

Reason of study:

- Follow up

Technique:-

- Approximately one hour after the IV injection of the radiopharmaceutical, whole body PET-CT was acquired from skull base to mid-thigh with the arms extended above the head.
- CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- **Radiopharmaceutical:** F-18-FDG 5 mCi injected I.V.

Weight:	68 Kg	Injection Site	Right hand
Height:	170 Cm	IV Contrast	No
Bl.Gluc.level	106	Biodistribution Time	60 min